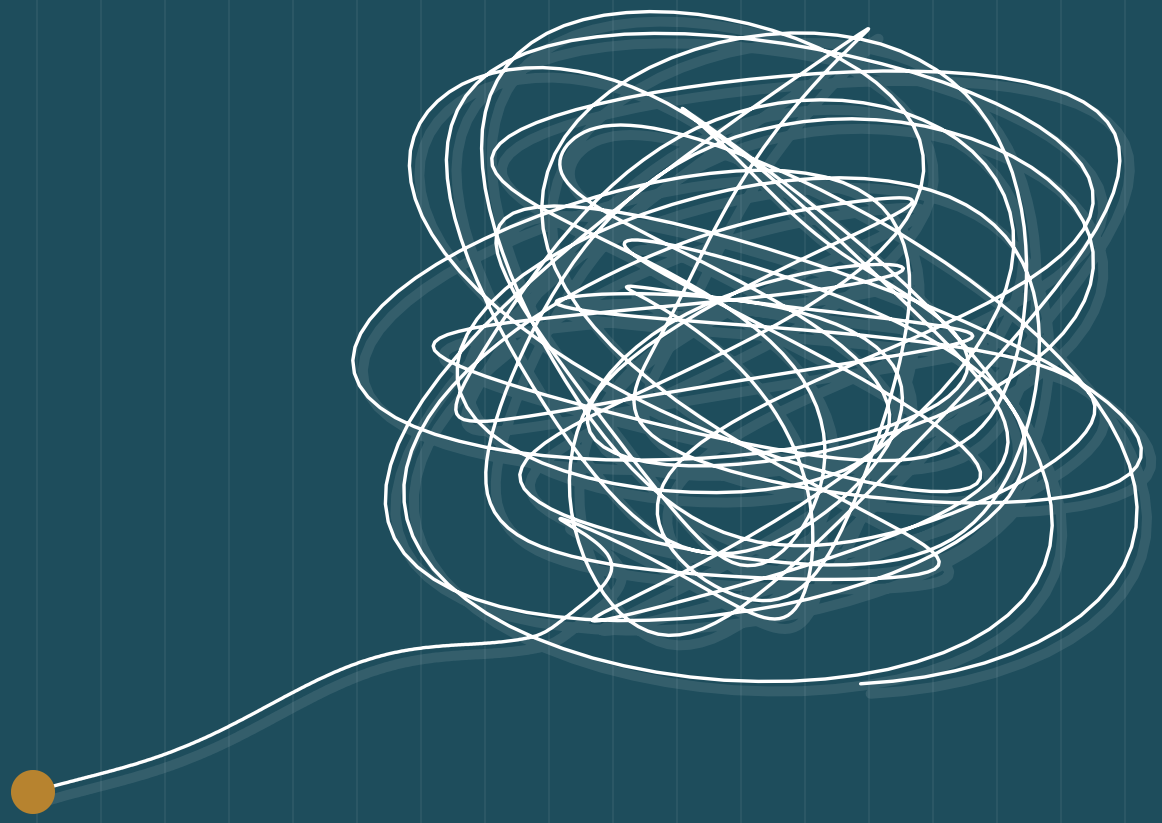
ANCHORED INSIDE SPECIALTY SERIES · NO. 1



A CLIENT & CLINICIAN WORKBOOK

Loosening the Grip

Breaking the OCD cycle with Exposure and Response Prevention

A practical workbook on understanding obsessions and compulsions, building an exposure ladder, resisting rituals, living with uncertainty, and staying well.

OCD | ERP | CBT | MINDFULNESS

Jessica Prosser, LPC

Independent resource in original language. Informed by established ERP and CBT approaches to OCD; see Section 15 for sources. Not affiliated with or endorsed by any cited author or publisher.

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SECTION 1

01 Before You Begin

This workbook is about changing your relationship with OCD, not winning an argument with it.

If you are here, OCD has probably been taking more from you than you want to give: time, peace of mind, maybe relationships or plans. You may have spent a long time trying to think your way out of doubt, only to find the doubt comes back louder. That is not a personal failing. It is how OCD works, and it is also why the way out looks different from what most people expect.

This workbook is built on **Exposure and Response Prevention (ERP)**, the most researched and effective psychological treatment for OCD, along with supporting tools from cognitive therapy, mindfulness and acceptance. In plain language: you will gradually and deliberately face what OCD tells you to avoid, while practicing *not* doing the rituals that keep it going. Over time, your brain learns that you can handle uncertainty and discomfort, and OCD's alarms lose their power.

KEY IDEA

What this workbook offers

- A clear picture of how OCD works, so it stops feeling mysterious or like a reflection of who you are.
- Step-by-step tools to map your OCD and build a personal exposure plan.
- Practical strategies for resisting compulsions, including the hidden ones that happen in your head.
- Ways to stay steady with thoughts and feelings you cannot control.
- Worksheets, logs and reflection prompts you can bring to session.

TRY THIS

How to use it

- Read in order. Sections 2 through 6 lay the groundwork for the practice sections that follow.
- Work with a therapist if you can. ERP is most effective with guidance, especially at the start.
- Go at a steady pace. A little practice every day beats a big effort once a week.
- Expect discomfort. Feeling anxious during practice is a sign you are doing it, not a sign it is failing.

SAFETY

Going gently

Facing fears on purpose is hard, and it is meant to be challenging but manageable. If you feel overwhelmed, slow down and talk with your therapist about adjusting the plan. OCD thoughts, even frightening ones, are not a sign that you are dangerous. If you are having thoughts of ending your life, or you are in crisis, call or text **988** (Suicide and Crisis Lifeline, U.S.) or your local emergency number. This workbook is educational and is not a substitute for professional care.

PAUSE AND NOTICE

As you begin, what would you most like OCD to stop taking from you?

SECTION 2

02 Understanding OCD

OCD is a pattern of unwanted thoughts and the urges to make them go away. It is not who you are.

Obsessive-compulsive disorder has two parts that feed each other. **Obsessions** are unwanted, intrusive thoughts, images, urges or doubts that cause distress. **Compulsions** are the things a person does, physically or mentally, to reduce that distress, prevent something bad from happening, or feel "just right."

OCD tends to target what matters most to you. A loving parent may have frightening thoughts about harming their child. A person who values honesty may worry they have lied. A person of deep faith may fear they have offended God. This is one of the cruelest features of OCD, and one of the most reassuring facts about it: **the thoughts upset you precisely because they go against who you are.**

Common themes

Theme	What the obsession sounds like	Common compulsions
Contamination	"What if I get sick, or make someone else sick?"	Washing, cleaning, avoiding touch, changing clothes
Harm	"What if I hurt someone, or already did?"	Avoiding knives or driving, checking, retracing steps
Checking and responsibility	"What if I left the stove on and the house burns down?"	Checking locks, appliances, emails; asking others to check
Taboo or unwanted thoughts	Unwanted sexual, violent or blasphemous thoughts	Mental reviewing, praying, confessing, avoiding people
Scrupulosity	"Did I sin? Did I do something morally wrong?"	Confessing, excessive prayer, seeking reassurance
Symmetry and "just right"	"It doesn't feel right. Something bad will happen."	Arranging, repeating, counting, redoing
Relationship	"Do I really love my partner? What if they're not the one?"	Comparing, testing feelings, seeking reassurance
Health	"What if this symptom means something serious?"	Body checking, researching, doctor visits, reassurance

OCD can attach to almost anything. Other themes include lucky or unlucky numbers; **sensorimotor** focus (becoming stuck noticing breathing, blinking or swallowing); **existential** or "am I real?" doubts; uncertainty about whether a memory is real or imagined; doubts about sexual orientation; unwanted sexual thoughts involving children, which are a common and deeply distressing OCD theme, not a sign of desire; and hoarding. When the theme changes, the pattern stays the same.

Two big families of OCD

However many themes there are, most obsessions lead back to one of two places when you follow them all the way down. One is **"something bad will happen"** (I'll get sick, someone will get hurt, I'll be punished). The other is **"it's just not right"**, a nagging feeling of wrongness or incompleteness with no clear disaster attached. Some people have both. It's like ice cream: there are endless flavors, but it's all still ice cream, and it all gets treated the same way.

Close cousins of OCD include body-focused repetitive behaviors (hair pulling, skin picking), body dysmorphic concerns, health anxiety, hoarding and some tic-related patterns. They overlap with OCD and often respond to similar tools.

KEY IDEA

Compulsion, habit or preference?

Many ordinary behaviors, like hand washing, locking up or saving money, are healthy. The question is not *what* you do but *what happens if you don't*. If you could skip it and shrug, it's a preference. If skipping it leaves you anxious, disgusted, stuck or unable to move on, it is probably working as a compulsion.

KEY IDEA

What you can and can't control

You don't get to choose which obsessions show up; that part belongs to OCD. You **do** get to choose what you do next. Compulsions feel automatic, but they are the part of the cycle you can change, and that's where all of this work happens.

KEY IDEA

The content isn't the point

It is tempting to focus on *what* the obsession is about. In OCD, the content matters far less than the pattern: an unwanted thought, a demand for certainty, and a ritual to get it. Notice the clue words. **"What if..."** usually signals an obsession. **"Just to make sure"** and **"just in case"** usually signal a compulsion.

KEY IDEA

What OCD is not

- It is not being tidy, organized or detail-oriented. Many people with OCD are none of those things.
- It is not a secret desire. Intrusive thoughts are unwanted by definition.
- It is not a character flaw or a lack of willpower. It is a recognized, treatable condition.
- It is not your fault, and it is not permanent in the way it may feel right now.

OCD often runs in families and involves differences in how certain brain circuits handle threat and certainty. Stress, life changes and hormonal shifts can make it flare. No single cause explains it, and you do not need to know why you have OCD to get better.

PAUSE AND NOTICE

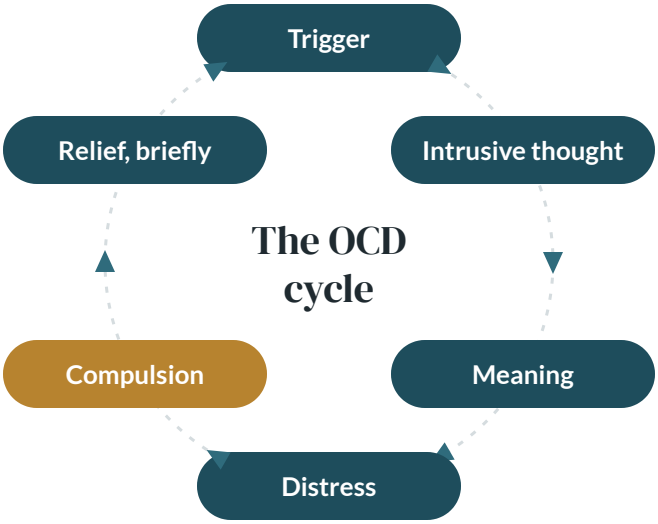
Which themes do you recognize in your own experience?

SECTION 3

03 The OCD Cycle

Compulsions bring relief in the moment, and that relief is exactly what keeps OCD going.

Everyone has strange, upsetting or random thoughts. Research consistently finds that most people experience intrusive thoughts that look a lot like OCD obsessions. The difference is not the thought itself. It is the *meaning* given to the thought and what a person does next.



1. **Trigger.** A situation, object, sensation or thought sets things off.
2. **Intrusive thought.** A doubt, image or urge pops up: *What if I didn't lock the door?*
3. **Meaning.** OCD treats the thought as important or dangerous: *If I don't check, it will be my fault if we're robbed.*
4. **Distress.** Anxiety, guilt, disgust or a sense that something is "not right" rises.
5. **Compulsion.** You check, wash, ask, review, avoid or neutralize to bring the distress down.
6. **Relief, briefly.** The distress drops, and the brain learns: *that ritual kept us safe.* Next time, the thought feels more urgent and the urge to ritualize is stronger.

KEY IDEA

The trap of relief

Each compulsion teaches your brain two things: that the thought was dangerous, and that you could not have handled it without the ritual. ERP works by interrupting that lesson. When you stay with the discomfort without ritualizing, your brain gets a chance to learn something new.

TRY THIS

A picture to remember

Think of OCD like a stray animal that shows up at your door. Every ritual is a little food. It leaves for a while, satisfied, and you have made sure it will come back hungrier. The more you feed it, the more often it returns. Response prevention means you stop feeding it, even when it complains loudly.

If animals don't work for you, think of a mosquito bite. Scratching feels like relief, but the more you scratch, the redder and itchier it gets. Leave it alone, and it itches for a while, then fades.

Why pushing thoughts away backfires

Try this: for the next ten seconds, do *not* think about a slice of chocolate cake. Most people find the cake shows up right away. Trying to suppress a thought tells your brain the thought is important, so it keeps bringing it back, like a dog that keeps dropping the ball at your feet because you reacted last time. Compulsions are really just attempts to control thoughts and feelings, and that's why they don't last.

Certainty is the bait

Underneath every theme is the same demand: *be 100% sure*. Most people go through the day assuming they are safe unless there's a clear sign of danger. OCD flips that rule: *assume it's dangerous unless you can prove it isn't*. Since almost nothing in life can be proven 100%, there's always room for doubt, and OCD lives in that room.

Avoidance counts too

Avoiding triggers works like a compulsion in disguise. Skipping the kitchen knives, taking the long way to avoid a certain road, or not touching door handles all prevent distress in the moment and shrink your world over time.

WORKSHEET

My OCD cycle

Fill in a recent example.

Step	My example
Trigger	
Intrusive thought	
What OCD said it meant	
Feelings (and how strong, 0 to 100)	
What I did to feel better	
What happened afterward	

PAUSE AND NOTICE

What has OCD's relief cost you over time?

SECTION 4

04 Thoughts Are Not Facts

Having a thought does not make it true, important or likely. OCD depends on believing otherwise.

OCD is often described as a "doubting disease." It thrives on a few predictable thinking patterns that make ordinary intrusive thoughts feel like emergencies. Learning to spot these patterns does not make the thoughts stop, but it can make them easier to step back from.

Thinking trap	What it sounds like	A different way to see it
Thoughts are important	"If I keep thinking this, it must mean something."	The more you try not to think something, the more it shows up. Repetition is a sign of effort, not truth.
Thinking equals doing (likelihood)	"If I think about an accident, it is more likely to happen."	Thoughts cannot cause events in the world.
Thinking equals doing (moral)	"Having this thought makes me as bad as someone who acts on it."	A thought is not an action, a wish or a reflection of character.
Overestimating danger	"Something terrible is going to happen."	OCD focuses on what is possible, not what is probable.
Too much responsibility	"If anything goes wrong, it will be my fault."	Responsibility is shared, and no one can prevent every possible harm.
Needing certainty (the core trap)	"I have to know for sure."	Certainty is not available about most things in life. Everyone lives with some doubt.
Needing control of thoughts	"I should be able to stop these thoughts."	No one controls what thoughts pop up. We choose how we respond.
Perfectionism and "just right"	"It has to feel exactly right before I can stop."	"Good enough" is a skill OCD has taken away. It can be practiced again.
All-or-nothing and "shoulds"	"If I'm not completely sure, I'm not sure at all." "I should never have a bad thought."	Most of life lives in the gray. Rigid rules feed OCD.
Catastrophizing	"If this happens, it will be unbearable and ruin everything."	People cope with far more than OCD predicts.

KEY IDEA

Why arguing with OCD doesn't work

With OCD, it can feel like stepping on the brakes while the brake line is cut: you know the thought is illogical, but the car doesn't stop. Logic doesn't stick when anxiety is high. Once you have heard the facts one time, going over them again becomes reassurance. A better response is short and the same every time: *"That's OCD."*

KEY IDEA

Possible versus probable

OCD asks, "Could this happen?" and the answer is almost always, "It's possible." A more useful question is, "How likely is it, really, and could I cope if it did?" Even then, the goal is not to prove OCD wrong. It is to stop needing to.

TRY THIS

Naming the trap

When an obsession shows up, try labeling it instead of answering it: *"That's the needing-certainty trap again."* or *"There's OCD, asking for a guarantee."* Naming creates a little space between you and the thought.

WORKSHEET

Catching thinking traps

Use this when you notice a strong urge to ritualize.

Intrusive thought	Thinking trap(s)	What I can remind myself

PAUSE AND NOTICE

Which thinking trap shows up for you most often?

SECTION 5

05 Mapping Your OCD

You cannot change a pattern you cannot see. A clear map turns OCD from a fog into something you can work with.

Before building an exposure plan, it helps to get specific. Many people are surprised by how many small rituals and avoidances they have stopped noticing because they have become routine.

Compulsions come in many forms

Type	Examples
Physical rituals	Washing, cleaning, checking, arranging, tapping, repeating actions, redoing tasks
Mental rituals	Reviewing memories, analyzing, praying in a set way, counting, replacing a "bad" thought with a "good" one, mentally checking your feelings
Reassurance seeking	Asking others, searching online, confessing, asking "Are you sure?", checking with a doctor again
Avoidance	Avoiding places, objects, people, words, media or situations that trigger the obsession
Safety behaviors	Using a tissue on door handles, keeping hands in pockets, only driving with someone else

WORKSHEET

My OCD inventory

List what you notice over the next week. Rate distress from 0 (none) to 100 (the most you can imagine).

Trigger	Obsession	Compulsion or avoidance	Distress (0 to 100)	Time per day

KEY IDEA

Family accommodation

OCD often pulls in the people around you: answering the same question many times, washing in a certain way, avoiding certain words, or doing checks for you. This comes from love, and it keeps OCD going. Section 13 covers how loved ones can help differently.

PAUSE AND NOTICE

What would you do with the time and energy OCD currently uses?

SECTION 6

06 How ERP Works

ERP doesn't teach you that bad things will never happen. It teaches you that you can handle uncertainty and discomfort.

Exposure means deliberately and gradually approaching the thoughts, images, objects and situations that trigger your obsessions. **Response prevention** means choosing not to do the compulsions, avoidance or reassurance seeking that usually follow. Together they give your brain new experiences that OCD's rules have been blocking.

What you learn through ERP

- That the feared outcome usually doesn't happen, or is far less likely than OCD predicts.
- That anxiety, disgust and "not right" feelings rise and fall on their own, without rituals.
- That you can do things you care about even when uncertainty is present.
- That you are stronger than OCD's alarms, and the alarms do not need to be obeyed.

Distress often drops during or across practices, but it doesn't have to drop for an exposure to be successful. What matters most is staying with the experience and resisting the ritual. Each time, you are teaching your brain a new rule: *I can feel this and still be okay.*

KEY IDEA

Two ways exposure helps

- **Getting used to it:** with repetition, a trigger often stops setting off such a strong alarm.
- **Learning you can handle it:** even when anxiety stays high, you learn that you can feel it, not ritualize, and be okay. Your ability to carry discomfort grows. This second kind of learning is why an exposure can be a success even if you feel anxious the whole time.

What happens to anxiety

Many people believe that if they don't do the ritual, their anxiety will climb forever. It can't. Anxiety is like a ball thrown into the air: however hard you throw it, it comes back down if you let it. Compulsions keep catching the ball on the way up, so you never get to see it land. When you face a trigger without the ritual, distress usually rises, peaks and then eases on its own. With enough repetition, that trigger stops setting off the alarm at all.

Sometimes the feared thing actually happens: you touch a doorknob and catch a cold that week. That isn't failure. Living with OCD was never about proving it wrong. It was about learning that you can handle uncertainty, take ordinary risks, and cope with what comes.

KEY IDEA

Pictures to keep in mind

- **The finger trap:** the harder you pull away, the tighter it grips. Moving toward it is what loosens it.
- **Crossing the river:** your free life is on the other bank. There is no bridge or boat, only a cold, murky river. The only way across is through.
- **The cold pool:** you can go in a toe at a time or cannonball in. Either way, you get used to the water if you stay in.

The simple rule: do the opposite

When OCD says *do something* (check, wash, ask, review), don't do it. When OCD says *don't do something* (touch it, go there, say it, think it), do it. You can't control which thoughts show up. You can control what you do next.

Common worry	What's actually true
"ERP means facing my worst fear on day one."	Exposures start with manageable steps and build gradually. You help decide the plan.
"If I stop ritualizing, my anxiety will never go down."	Anxiety may spike at first, then it usually eases as your brain learns the alarm was false.
"Exposure might make the bad thing happen."	Exposures are designed to be safe. They target fear, not real danger.
"I'll be anxious forever if I don't get certainty."	Uncertainty becomes more tolerable with practice. You can learn to carry doubt lightly.
"I tried exposure and it didn't work."	Hidden rituals, reassurance or skipping response prevention are common reasons. A therapist can help adjust.

Ways to practice

- **Planned (proactive) practice:** you choose an exposure on purpose, like serving the ball.
- **Everyday (reactive) practice:** when OCD shows up on its own during the day, you meet it without ritualizing, like returning a serve.
- **Step by step or all in:** some people climb a ladder gradually; others take on harder steps sooner. Decide with your therapist.
- **Make it a game:** keep score of "me versus OCD," take a daily challenge, or bring humor and play into practice.
- **Stand tall:** sitting or standing with open, confident posture during exposures tells your brain the thought isn't a threat.

Exposures sometimes go a little beyond what most people would do, on purpose, such as touching a floor and then eating a snack. The aim isn't to live that way forever. Stretching past "normal" for a while helps you land in a balanced, flexible middle.

Think of a pendulum held up high on the OCD side. When you let it go, it doesn't stop neatly at "normal." It swings past the middle before it settles. The stronger your OCD, the further past normal your practice may need to go, and the further you go, the better protected you are against OCD creeping back.

KEY IDEA

Rule number one: no new OCD

As you push back, OCD will try to change the rules: a new ritual, a new "just to be safe," a new thing to avoid. Expect it. OCD often gets louder right when you stop giving in, the same way someone kicks a vending machine that stops handing out sodas. That surge is a sign the work is reaching it. The rule is simple: **no new OCD.**

How change usually shows up

1. You start trying to resist more often.
2. You succeed at resisting more often.
3. The distress around triggers eases.
4. Last of all, and sometimes only partly, the obsessions show up less. Even when they keep visiting, they lose their grip.

KEY IDEA

The willingness mindset

ERP asks for willingness, not bravery. You don't have to feel ready or confident. You only need to be willing to feel uncomfortable for something that matters to you.

PAUSE AND NOTICE

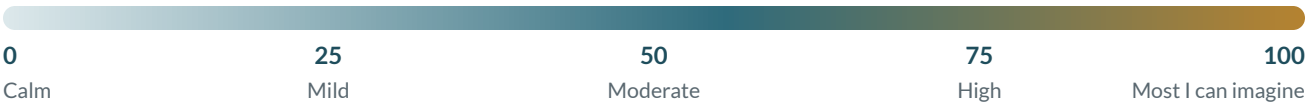
What would make the discomfort of ERP worth it for you?

SECTION 7

07 Building Your Exposure Ladder

A good ladder starts low enough to be doable and climbs toward the things OCD has taken from you.

An exposure ladder (sometimes called a hierarchy) is a list of situations that trigger your OCD, ranked from least to most distressing. You will use the **SUDs scale** (Subjective Units of Distress) to rate each one from 0 to 100.



Three kinds of exposure

Type	What it means	Examples
In real life (in vivo)	Facing real situations, objects or cues that are realistically safe	Touching a doorknob, leaving the house after one lock check, a video of a storm instead of a real one
Body sensations (interoceptive)	Bringing on feared physical sensations on purpose	Spinning to feel dizzy, noticing your heartbeat, focusing on swallowing or blinking
In imagination (imaginal)	Facing feared thoughts or outcomes that can't be faced in real life	Writing and listening to a script about a feared outcome (Section 10)

For any exposure to work, it needs enough **intensity** (it should feel challenging), enough **time** (stay with it), and enough **repetition** (practice often). Remove safety behaviors and comforts that signal danger, like gloves or a "safe" person nearby, as you are able.

How to build it

1. Using your inventory from Section 5, list 10 to 15 situations that trigger your obsessions or that you avoid.
2. Rate how much distress each would cause if you faced it without ritualizing (0 to 100).
3. Rate how much you currently avoid it (0 to 100).
4. Add variety: different places, times, people and levels of difficulty. Include imaginal exposures for thoughts that can't be faced directly.
5. Choose a first step with distress around 30 to 40, challenging but manageable.

Example: fear of contamination

Situation (without washing afterward)	Distress	Avoidance
Touch the inside of my own front door handle	30	35

Situation (without washing afterward)	Distress	Avoidance
Touch a grocery cart handle	45	50
Touch a public door handle, then touch my face	60	70
Sit on a public bench, then make a snack	75	80
Touch a public restroom door, then hug my child	90	100

Exposure idea bank

Use these to spark ideas. Pick what fits your fears and your plan with your therapist. Exposures should be realistically safe, legal and in line with your values. They target fear, not real danger.

Theme	Exposure ideas
Contamination	Touch "dirty" surfaces, then your face, hair and clothes; eat a snack without washing; spread the "contamination" to your bed, couch and phone so there are no clean safe zones
Harm and responsibility	Use knives to cook near others; drive over bumps without circling back; leave a candle or appliance on and step outside; look at news or images on your theme; say trigger words
Health	Read stories about the illness you fear; skip body checks and symptom searches; eat a food you've labeled "risky" (with medical guidance where needed)
Just right and symmetry	Leave things uneven; step on one crack and not the other; put a frame slightly crooked and walk away
Numbers	Do things the "wrong" number of times, below the magic number; write or wear the unlucky number
Scrupulosity and morality	Pray without the ritual or "mess up" the words; break rigid self-rules in small ways; involve a trusted faith leader in planning when helpful
Unwanted sexual or relationship thoughts	Do the everyday things OCD has made off-limits, like bathing your child or sitting at the park, without checking your reactions; stop testing your feelings about your partner
Rereading and rewriting	Read aloud while covering each line as you go; set a timer per page; write a quick draft with no editing and fix it later
Body sensations	Breathe through a thin straw, spin in a chair, chug water and jump (sloshy stomach), stare at yourself in a mirror for two minutes (for "am I real?" fears)

TRY THIS

Layer it like a cake

Exposures get stronger when you combine them. Pair a real-life trigger with a body sensation and say your feared thought out loud at the same time. For example: listen to vomiting sounds, bring on a gag, and repeat, *"I'm going to throw up."*

TRY THIS

Make it playful

Exposure doesn't have to be grim. Turn trigger words into a card-matching game, say your script in a silly accent or an opera voice, or eat candy after touching the floor. Humor isn't making fun of your fears; it's taking away their authority.

TRY THIS

Keep it flexible

Your ladder is a guide, not a contract. You don't have to climb one rung at a time in perfect order. Mixing easier and harder exposures, and practicing in different settings, can make learning stronger.

WORKSHEET

My exposure ladder

Start at the bottom. Add rungs as you go.

Situation or exposure	Ritual I'll resist	Distress (0 to 100)	Avoidance (0 to 100)

SECTION 8

08 Response Prevention

Exposure opens the door. Not doing the ritual is what lets new learning in.

Response prevention means choosing not to do the things OCD demands. It is often the hardest part of ERP, and the most important. Without it, exposure can turn into a ritual of its own. Think of it as non-negotiable: the exposure sets up the learning, and skipping the ritual is what lets it happen.

Ways to loosen the grip

Strategy	How it works	Example
Delay	Postpone the ritual for a set time. Often the urge passes.	"I can check the stove in 15 minutes." Then extend the delay.
Shorten	Do less of the ritual.	Wash for 20 seconds instead of 2 minutes.
Change it	Alter the ritual so it loses its "rightness."	Check once with the wrong hand, or out of order.
Limit it	Set rules in advance.	One shower a day; no washing after touching mail.
Make it harder	Add obstacles to the ritual.	Put the hand sanitizer in the car; leave the phone in another room.
Stop it	Choose not to do it at all.	Leave the house after locking up once, without returning.

Hidden rituals to watch for (mental ones are sneaky)

- **Reassurance seeking:** asking loved ones, googling, confessing, or asking "Are you sure?"
- **Mental reviewing:** replaying events to make sure nothing bad happened.
- **Neutralizing:** replacing a bad thought with a good one, praying it away, or saying a phrase.
- **Subtle avoidance:** distracting yourself during an exposure or doing it "carefully."
- **Checking feelings:** testing whether you still feel anxious, aroused, in love or "right."
- **Fixing it mentally:** replaying a scene so it ends well, rescuing someone in your imagination, or reaching for a positive thought to cancel a bad one.
- **Figuring it out:** analyzing, problem-solving or planning to *make sure* nothing bad happens. Even worrying itself can be a ritual.
- **Beating yourself up:** self-criticism or rumination used to prove you're a good person or to "pay" for a thought.
- **Helpful habits used compulsively:** gratitude lists, meditation, list-making or even exposures done to make the anxiety go away.

Label it and shift

When an obsession shows up, give it a name, *"That's OCD"*, and gently move your attention back to what you were doing. Treat it like a toddler who swears to get a reaction: the less of a reaction it gets, the sooner it loses interest. The long-term goal is to notice the thought, shrug, and keep going.

KEY IDEA

If you can't resist all of it

Aim for the **least amount of OCD** that lets you keep moving. If you can't sit at the family table without a towel on the chair, sit there with the towel for now. Then work on removing the towel. Every bit you trim is a win.

TRY THIS

Speed bumps and tokens

- Put obstacles between you and the ritual: remove the security cameras, leave the sanitizer at home, turn off symptom-checking apps, or agree to call your therapist before any non-routine doctor visit.
- Ration reassurance: start the day with a few tokens (poker chips work well). Each question costs one. When they're gone, they're gone. Shrink the number over time.
- Stretch the gap: can you go five minutes without checking? An hour? A day?

KEY IDEA

Responding without answering

When OCD demands an answer, you can respond in a way that accepts uncertainty: *"Maybe, maybe not." "I'm not going to figure this out right now." "That's possible, and I'm choosing to live with not knowing."* The goal is not to win the argument. It is to stop having it.

WORKSHEET

My response prevention plan

Choose one or two rituals to work on first.

Ritual	My strategy	What I'll do instead	Who can support me

PAUSE AND NOTICE

Which of your rituals is the hardest to notice, and how will you catch it?

SECTION 9

09 Doing an Exposure

The goal isn't to feel calm. It's to stay with the discomfort long enough for your brain to learn something new.

Before

- Choose a step from your ladder. Decide exactly what you will do and which rituals you will resist.
- Write down what OCD predicts will happen and how strongly you believe it (0 to 100%).
- Rate your starting distress.

During

- Approach the trigger fully. Notice thoughts, feelings and sensations without trying to push them away.
- Resist every ritual, including mental ones and subtle avoidance.
- If you want, rate your distress every few minutes, but don't wait for it to drop before you stop.
- Stay long enough to learn something, often 20 to 45 minutes, or until the urge to ritualize has eased.

After

- Compare what happened with what OCD predicted.
- Notice what you learned about yourself and your ability to cope.
- Keep resisting rituals afterward. Washing or reviewing later can undo the work.
- Repeat the same exposure often, in different places and ways, before moving up.

KEY IDEA

During practice, skip the calming tools

Breathing exercises, soothing self-talk or distraction during an exposure can quietly act as safety behaviors. The anxiety will come down on its own. Bravery statements ("OCD wants me to stop, and I'm staying") are fine. Reassurance ("nothing bad will happen") is not.

TRY THIS

Check in with yourself

- What am I noticing in my body right now?
- What is OCD telling me?
- If it's getting easier, how can I turn it up a notch?
- Am I still fully in it, or have I started drifting, distracting or doing it "carefully"?

Count it as a win when you were **willing**, when you stayed with discomfort and didn't ritualize. Whether your anxiety came down that day is not the measure. If an exposure turns out much harder than expected, stay with it as best you can, then make the next step a little smaller. Don't quit the ladder.

TRY THIS

Coping statements

- "This is uncomfortable, and I can do uncomfortable things."
- "I'm practicing living with uncertainty."
- "Anxiety is a feeling, not a fact."
- "I'm choosing what matters to me over what OCD wants."

How much practice is enough?

Like going to the gym, results come from dose. That means hard enough (a challenging step teaches more than an easy one), long enough (a one-second touch isn't enough), and often enough. Most of the progress in ERP happens between sessions, in the daily practice you do on your own. Plan proactive practice once or more a day. Avoid scheduling it right before bed unless your rituals happen at bedtime.

WORKSHEET

Exposure practice log

Copy this page as needed.

Date	Exposure	What OCD predicted	Distress start / peak / end	Rituals resisted?	What I learned

SECTION 10

10 Imaginal Exposure

Some fears can't be faced in real life, but they can be faced on paper and in your mind.

When OCD centers on thoughts, future events or things that can't safely or ethically be recreated (fears of harming someone, going to hell, becoming ill), **imaginal exposure** lets you face the feared thought directly. You write a short story about the feared outcome and read, record or listen to it repeatedly without neutralizing.

Writing a script

1. Write in the first person and present tense: "I am..."
2. Include the feared thought or outcome and, importantly, the uncertainty: *I'll never know for sure.*
3. Add details about feelings and consequences that OCD threatens.
4. Keep it about a page. Start with a less intense version and build from there.
5. Read or listen to it daily until it becomes boring or much less distressing.

A four-part script formula

1. Trigger	2. Fear or uncertainty	3. Push back on OCD	4. Willingness statement
I can't remember if I locked the back door.	Someone could get in, and I may never know whether it was my fault.	OCD wants me to drive home and check, but I'm not going to.	Because I want my life back more than I want certainty.
My throat feels tight.	This could be something serious, or it could be nothing. I may never know for sure.	OCD wants me to search my symptoms online, but I won't.	Because searching has never actually given me peace.

Ways to make a thought lose its power

- **Repeat it** until it becomes dull, the way a word sounds strange and meaningless if you say it over and over, or gum loses its flavor.
- **Agree with it,** lightly: "Maybe so." Arguing gives it importance; agreeing takes the fight out of it.
- **Exaggerate it** until it becomes almost silly.
- **Lean into uncertainty:** "Maybe, maybe not. I'll never know for sure, and I can live with that."

Thought exposure (scripting)

Scripting is a short, focused form of imaginal exposure. Instead of a full story, you boil OCD's fear down to a sentence or two and say it, out loud, over and over.

TRY THIS

Two quick experiments

- Say the word *milk* once and notice what comes to mind. Now say it fast, over and over, for one minute. Most people find it turns into a meaningless sound.
- Imagine someone following you around for three days insisting the sky is yellow. Arguing doesn't stop them. What does? *"You're right, it's yellow."* Agreeing doesn't make it true. It just ends the argument.

Script type	What it sounds like	When to use it
Feared outcome	"I touched the railing, and I'm going to get seriously sick."	Usually the clearest place to start
Uncertainty	"Maybe I hit someone, maybe I didn't. I'll never know."	When the core of the fear is not knowing, or when the feared-outcome script slides into figuring things out

How to run a script

1. Agree on the script with your therapist. Keep it short.
2. Say it out loud, at a steady or quick pace, with open, upright posture.
3. Check in briefly every few minutes: what am I noticing? Then go straight back to the script.
4. Expect OCD to change the subject, add new details, or turn to *"you're a terrible person."* Bring yourself back to the script.
5. Keep going. Five minutes is short; meaning often fades somewhere between 5 and 20 minutes, and sometimes longer. If you've stayed with it a long time and it's still loud, that still counts. You practiced not obeying.

Use scripts in two ways: as **scheduled practice** (for example, a timed session once or twice a day) and as an **in-the-moment response** when OCD spikes and "That's OCD" isn't enough. Think of it like taking a pain reliever for a headache: when OCD hits hard, do the exposure. You can also pair a script with a real-life exposure, such as touching the doorknob while saying, *"Yep, now I'm contaminated."*

KEY IDEA

Watch the slide into shame

A script should feel like facing a fear, not like beating yourself up. If it turns into heavy, hopeless rumination and your body curls inward, switch to the uncertainty version or pause and talk it through with your therapist. If saying certain words conflicts with your faith, tell your therapist. Together you can check with a faith leader you trust and find another route.

KEY IDEA

Why it helps

Avoiding a thought makes it feel more dangerous. Deliberately thinking it, without rituals, teaches your brain that a thought is just a thought, even an awful one, and that uncertainty can be lived with.

SAFETY

Use with care

Imaginal exposure is best planned with a therapist, especially for harm, sexual or religious themes. It is not meant for trauma memories, and it should never involve real danger. If a script brings up suicidal thoughts or overwhelming distress, pause and contact your therapist or a crisis line.

WORKSHEET

My script plan

Question	My answer
The fear I'll write about	
The uncertainty to include	
Starting intensity (0 to 100)	
How often I'll practice	

SECTION 11

11 Mindfulness and Acceptance

You can't stop thoughts from arriving. You can change how tightly you hold them.

Mindfulness and acceptance tools support ERP by helping you notice thoughts without getting pulled into them. The aim is not to feel relaxed or make thoughts disappear. It is to let them come and go while you keep living.

Mindfulness simply means paying attention to the present moment without judging it. Picture your attention as a spotlight: it can rest on one thing at a time. OCD keeps swinging it to the past (*did I?*) or the future (*what if?*). Practice is learning to bring the spotlight back to right here. A thought isn't "bad" and a feeling isn't "bad"; those are judgments added on top.

TRY THIS

One-minute focus

Pick an ordinary object, like a pen. Study it for one minute. Each time your mind wanders, lift a finger, then come back. Notice new details: color, texture, light. Wandering isn't failing. Coming back is the exercise.

Ways to practice

- **Notice and name:** "I'm having the thought that I left the door unlocked." Adding "I'm having the thought..." creates distance.
- **Leaves on a stream:** picture each thought on a leaf floating by. You don't push it or hold it.
- **Trains at a station:** thoughts pull in and pull out. You don't have to board every train.
- **Anchor in the senses:** name five things you see, four you hear, three you can touch.
- **Treat it like a spam call:** you recognize the number, you don't pick up, and you go back to what you were doing. No need to argue with the caller.
- **Willingness:** "I don't like this feeling, and I'm willing to have it."

Watch for mindfulness becoming another ritual. If you rush to meditate *so that* the anxiety will go away, it has started working like a compulsion.

KEY IDEA

Self-compassion: the secret weapon

OCD often comes with a harsh inner critic. Criticizing yourself turns up the body's threat system; kindness calms it. Nobody asks for OCD, causes it on purpose, or controls which thoughts arrive, so there's no reason to punish yourself for having it. Speaking to yourself the way you would speak to a struggling friend makes it easier to keep practicing.

TRY THIS

A self-compassion break

1. **Notice:** "This is a hard moment. OCD is telling me scary things."
2. **Remember you're not alone:** "Lots of people live with this. I'm not the only one."
3. **Offer kindness:** a hand on your heart or a gentle word, such as *"Having OCD is hard, and I'm being brave."*

Values: what you're moving toward

Exposure is easier when you know what it's for. Values are the qualities you want your life to stand for, like being a present parent, a reliable friend, or a curious learner. When OCD pulls you away from your values, you can choose to move toward them anyway.

Values work like a compass direction, not a destination: you can always travel further east, and if you drift west, you can turn around. Many moments with OCD are a **choice point**. You can do what OCD wants or what your values want, but usually not both. A parent whose OCD demands a shower while their baby cries can choose to pick up the baby, "dirty" and all.

There's an old story about two wolves at war inside each of us, one fed by fear, the other by courage and care. Which one wins? The one you feed. Be aware that OCD can also borrow your values, turning care for your health, your family, your work or your faith into rules. Real values give you freedom. OCD's versions give you rituals.

WORKSHEET

My values compass

Area of life	What matters to me	How OCD gets in the way	One step toward it this week
Relationships			
Work or school			
Health and rest			
Fun and growth			

PAUSE AND NOTICE

What would you do this week if OCD weren't in charge?

SECTION 12

12 Working With Your Thoughts

Curiosity, not argument. The goal is to loosen OCD's beliefs, not to find certainty.

Cognitive tools can help when beliefs about thoughts, responsibility or danger feel especially stuck. They work best alongside ERP, not as a replacement for it. Be careful not to turn them into a new form of reassurance: if you notice yourself using these questions over and over to feel certain, that's OCD again.

KEY IDEA

A one-time rule

It's fine to get accurate information about a fear **once**, such as how an illness is actually spread. After that, revisiting the same question isn't learning. It's reassurance, and the answer to OCD becomes, "That's OCD." A warning sign: you challenge the same thought session after session, feel better briefly, and it's back the next week.

Questions to explore

- What would I tell a friend who had this thought?
- Is this thought about something possible or something probable?
- Has this thought ever caused something to happen?
- If someone else had this thought, would I think they were dangerous or bad?
- Who else shares responsibility here?
- What is the cost of believing this thought, and the cost of acting on it?

Behavioral experiments

A behavioral experiment tests an OCD belief through action. For example, if OCD says, *"If I think about my family getting sick, it will happen,"* you might deliberately think it for a set time each day and record what actually happens.

WORKSHEET

Thought record

Situation	Intrusive thought	What OCD says it means	Feeling (0 to 100)	A more balanced view

WORKSHEET

Behavioral experiment

Belief to test	My prediction (and % belief)	The experiment	What happened	What I learned

SECTION 13

13 Family and Loved Ones

Loved ones can be a powerful part of recovery when they support you rather than the OCD.

Family members and partners often get drawn into OCD out of love: answering the same question again, joining in rituals, avoiding certain topics. This is called **accommodation**. It eases distress in the moment and, like any compulsion, keeps OCD strong over time.

Accommodation	Supportive alternative
Answering the same reassurance question repeatedly	"I love you, and I'm not going to answer that one. I know you can handle not knowing."
Doing checks or washing on someone's behalf	Agree in advance which tasks you'll step back from, a little at a time.
Avoiding words, places or objects	Gradually return to normal routines as part of the ERP plan.
Getting frustrated or arguing with the OCD	Stay calm and warm. Separate the person from the OCD.

KEY IDEA

For loved ones

- Learn about OCD and ERP together.
- Reduce accommodation gradually and by agreement, not suddenly.
- Notice and praise effort, not just results.
- Keep life as normal as possible, and take care of yourself too.

What to say instead of reassuring

- *"That's OCD."*
- *"Maybe, maybe not."*
- *"We already talked about that one. I'm not going to feed the OCD."*
- *"I believe you can handle not knowing."*

Agree on these phrases ahead of time so they don't feel like rejection. Be warm toward the person and firm toward the OCD. Nobody in the family is qualified to give certainty anyway; a partner can't promise a mole is harmless, and a parent can't promise no one will throw up tonight.

KEY IDEA

Expect a burst before the change

When accommodation stops, OCD usually protests loudly at first: pleading, anger, tears, "you don't love me." Think of the vending machine that stops working: people shake and kick it before they walk away. Plan ahead for how you'll stay calm, step away if needed, and who you'll call for support. If a loved one talks about suicide, take it seriously and follow your safety plan or call 988 or emergency services, while still not restarting the accommodation.

Boundaries and encouragement

Boundaries are about what *you* will and won't do; they don't require the other person's agreement. For a loved one who isn't ready for treatment, families can still lovingly step back from accommodation ("I'd love for you to get help, and I'm not willing to keep doing things that make OCD stronger"). Parent-focused programs such as SPACE (Supportive Parenting for Anxious Childhood Emotions) show that changing how parents respond can reduce a child's OCD and anxiety even when the child isn't in therapy.

Rewards help too, for adults as well as kids. Tie them to **actions** people can control (doing an exposure, resisting a ritual, going to session), never to having thoughts or feelings. Small privileges, a favorite activity or a points chart all work. Celebrate small wins rather than brushing them off.

WORKSHEET

Stepping back from accommodation

Plan small steps, starting with the easiest. Note the support each person will need.

Step	What changes	Support we'll need
Where we are now		
Easy step		
Medium step		
Harder step		
Final goal		

WORKSHEET

Family agreement

Fill this in together.

What we'll change	What we'll say instead	Start date

SECTION 14

14 Staying Well

Recovery isn't the absence of intrusive thoughts. It's no longer letting them run your life.

Intrusive thoughts will still show up sometimes, especially during stress, illness or big life changes. That is normal. A **lapse**, a temporary slip back into rituals, is not a relapse. It is a signal to return to the tools that worked.

Keeping your gains

- Keep doing small exposures in daily life, even after formal practice ends.
- Treat new obsessions the same way: notice, label, and don't ritualize.
- Watch for early warning signs like increased avoidance, reassurance seeking or mental reviewing.
- Schedule booster sessions with your therapist when needed.
- Expect OCD to switch themes; it often moves like a whack-a-mole game. A new theme is still OCD and gets the same response.
- Start designing your own exposures. The goal is to become your own ERP coach.
- When OCD flares, ask: *"Who gets the point today, me or OCD?"* and choose one thing to do that OCD wouldn't like.
- Be kind to yourself. Self-criticism tends to fuel OCD; self-compassion helps you keep going.

WORKSHEET

My wellness plan

Question	My answer
My early warning signs	
Situations that may be harder for me	
Exposures I'll keep practicing	
What I'll say to myself after a lapse	
People I can reach out to	

PAUSE AND NOTICE

Looking back on this work, what are you most proud of?

SECTION 15

15 For Therapists

Clinical notes for using this workbook in treatment.

KEY IDEA

Assessment

- Combine a clinical interview and functional analysis with symptom measures: Y-BOCS (or CY-BOCS) symptom checklist and severity ratings, plus self-reports such as the OCI-R or DOCS. Repeat severity ratings regularly to track progress.
- Y-BOCS severity (0 to 40): 0 to 7 subclinical, 8 to 15 mild, 16 to 23 moderate, 24 to 31 severe, 32 to 40 extreme.
- Interviewing tips: listen for "what if" (obsessions) and "make sure" or "just in case" (compulsions); offer normalizing examples so clients can disclose taboo content; with youth or low insight, start by asking about compulsions.
- Map obsessions, overt and covert compulsions, avoidance, reassurance seeking and family accommodation.
- Assess insight (good/fair, poor, absent) and tic history, comorbid depression, anxiety, eating concerns and trauma, as well as cultural and religious context.
- Differential diagnosis: generalized anxiety and other anxiety disorders, autism-related routines and interests, behavioral addictions, and tics or Tourette's. Distinguish suicidal ideation from ego-dystonic suicidal obsessions through careful risk assessment.
- Explore motivation, valued life goals and prior treatment response, including medication history.

KEY IDEA

Treatment structure

- Typical course: roughly 12 to 22 sessions depending on severity, with tapering booster sessions.
- Sequence: assessment and psychoeducation, motivation and goals, cognitive and mindfulness skills, ERP, relapse prevention.
- Assign between-session practice after every session, usually daily.
- Collaborate on the ladder; begin near 30 to 40 SUDs and vary exposures across contexts.
- Gradually shift responsibility for designing exposures to the client (or parents, for youth).

KEY IDEA

Clinical considerations

- Frame exposures around expectancy violation and tolerance of uncertainty rather than habituation alone.
- Watch for subtle avoidance, distraction, mental neutralizing and reassurance within sessions, including from the clinician.
- Use relaxation and coping skills only as a short-term bridge or to build tolerance for ERP; used during exposures, they can function as safety behaviors.
- Troubleshooting: address resistance to the rationale, catch subtle avoidance or reassurance, and adjust when an exposure proves more intense than expected. Check dose (intensity, duration, frequency) and between-session adherence.
- Motivation (wanting change) and adherence (doing the work) are different targets; values work, self-compassion and contingency management can support both.
- For youth, structured behavior plans (Premack/first-then, point or level systems, response cost) coordinated with caregivers can support ERP, alongside a graded plan for withdrawing family accommodation.
- Use cognitive work (Socratic dialogue, thought records, behavioral experiments) to support engagement, not to provide certainty.
- Combined CBT and medication (SRIs) is often indicated for severe OCD or significant comorbidity. As a non-prescriber, educate, support informed decisions, monitor response and address non-adherence; coordinate with prescribers.
- Consider a higher level of care (intensive outpatient, partial hospitalization, residential or intensive ERP programs) when weekly outpatient treatment isn't enough.
- For treatment-resistant presentations, some clinicians integrate EMDR to address earlier experiences linked to OCD onset. Evidence is still emerging; consider it an adjunct to, not a replacement for, ERP.

KEY IDEA

Course notes: assessment in practice

- Run the Y-BOCS as a semi-structured conversation, not a script read verbatim. Rephrase sensitive items with normalizing lead-ins (for example, "Everybody thinks about sex, but sometimes people have sexual thoughts that really upset them..."), ask about faith before scrupulosity items, and ask about orientation doubts in both directions. Allow one or two sessions; assessment doubles as psychoeducation.
- Set the tone early: make it clear nothing will shock you, and keep a calm, matter-of-fact face. Say disturbing content plainly, as if reading a grocery list, to model that it isn't a credible threat.
- Useful compulsion probes: What do you do to feel okay? To keep the bad thing from happening? To get rid of the thought? To stay safe?
- Severity items are rated 0 to 4 (time, interference, distress, resistance, control) for obsessions and compulsions. For obsessions, frame resistance as "tuning it out" rather than suppressing. With low insight, rate the five items for OCD as a whole and double the score.
- The goal is to get into, and stay in, the mild range (about 8 to 11), not zero. Expect change in this order: effort to resist, then success at resisting, then distress, and last, obsession frequency.
- When a client won't name the thought (saying feels like doing), narrow it down with yes/no categories and lean on the checklist to show it's common. You can also work from triggers, avoidance and response prevention; details often come out during exposure.

KEY IDEA

Course notes: differentials and comorbidity

- GAD: worry about everyday matters and worry used as the control strategy. OCD: ego-dystonic, doesn't respond to logic, ritualized, centered on uncertainty. Treat co-occurring anxiety through an exposure lens.
- Autism: special interests are enjoyed (ego-syntonic); obsessions are unwanted. Addictions and BFRBs: ask what drives the behavior (doubt and checking vs. pleasure or relief). OCPD: rigid and ego-syntonic ("if everyone did it my way...").
- Tics: urge release vs. "I have to, or else." Tic-related OCD gets ERP; tics themselves get habit reversal or CBIT.
- Trauma: OCD may develop after trauma as a bid for control, or trauma may feed an existing theme. Treat OCD with ERP and trauma with PE or CPT, or refer for EMDR, coordinating so that self-soothing advice doesn't become compulsive.
- Relationship OCD vs. anxious attachment, and "just right" vs. perfectionism: the label matters less than the function. Is the behavior seeking certainty?
- Suicidal obsessions vs. ideation: one is a feared, unwanted thought; the other is an option being considered. Both can co-occur. Treating an obsession as a credible threat (for example, a safety plan that asks the client to report every thought) can create a confessing compulsion; ideation calls for standard risk management.
- Substances such as alcohol, cannabis or benzodiazepines during exposures blunt new learning. Address use first or in parallel (dual-diagnosis care).

KEY IDEA

Course notes: running ERP

- Give corrective information once; after that, "That's OCD." Tell clients ahead of time that you'll respond this way, and be warm to the person and firm with the OCD.
- Groundwork usually takes about three to four sessions (education, rationale, hierarchy) before active exposure. Promise collaboration: every exposure is agreed on, no surprises unless planned, and you'll do it alongside them.
- In session, stay calm and lean in without reassuring. Check in briefly and often ("What's your level? What's OCD saying?"), raise intensity when distress dips, and be directive about subtle avoidance ("Look at the hardest part").
- SUDS ratings are optional. Drop them if they become perfectionistic, and watch for exposure itself being used to make anxiety go away. Reinforce willingness and courage, not anxiety reduction.
- Work across themes by difficulty level rather than finishing one theme before starting the next, since OCD tends to migrate.
- When an assigned exposure starts functioning as reassurance ("my therapist wouldn't assign anything truly dangerous"), hand the design to the client.
- After a refused exposure or a rupture, own your part, repair, and re-plan with smaller steps or stronger incentives. Avoid a battle of wills.
- Don't continue indefinitely without active ERP. Months of talk without change implies OCD is untreatable; refer when needed.
- Ethics: exposures confront content the client's own mind already produces. Never introduce anything illegal or developmentally inappropriate, and find out exactly what a child pictures before planning.

KEY IDEA

Course notes: motivation, families and dose

- Treat motivation (wanting change) and adherence (doing it) as separate targets. Competing short-term relief and long-term goals are normal. Build motivation with values, "what happens if nothing changes," and the "magic wand" question.
- Contingency management: reinforce and consequence controllable behaviors only. Options include praise, prizes, privileges, planned ignoring, loss of privileges, job grounding, response cost, Premack (first-then), points and level systems. Increase structure as severity rises rather than backing off.
- For treatment refusal, coach families in strategic pressure (systematically and supportively withdrawing accommodation; Pollard) or parent-only SPACE (Lebowitz). Prepare them for an extinction burst and plan how to handle threats.
- Dose: moderate OCD often needs about twice-weekly sessions (90 minutes can help); severe, about three times a week; severe to extreme, daily, meaning IOP or residential. Residential OCD programs focus on ERP and ACT.
- Medication: SSRIs are first line; OCD often needs higher doses and 8 to 12 weeks at a therapeutic dose to judge response. Augmentation and clomipramine are options for prescribers. Combined treatment often helps most, particularly in severe cases or youth. Benzodiazepines are generally not recommended. Leave dosing decisions to the prescriber.
- Emerging approaches include inference-based CBT, OCD-specific TMS and psilocybin research. ERP remains the first-line psychological treatment. Telehealth ERP is effective and makes home-based exposures easy to observe.

SAFETY

Cautions

Differentiate ego-dystonic intrusive harm thoughts from genuine risk through careful assessment. Screen for suicidality, which is elevated in OCD, and keep a safety plan where needed. Imaginal exposure is not appropriate for processing trauma memories. Avoid providing reassurance about obsessional content in session.

SOURCES AND FURTHER READING

This workbook is an independent resource written in original language. It draws on established cognitive behavioral approaches to OCD, including Exposure and Response Prevention, and was informed by: Hyman, B. M., & Pedrick, C., *The OCD Workbook* (New Harbinger); Auguste, M., *Overcoming Your OCD: A Therapy Assignment Workbook* (Between Sessions Resources, 2018); Wilhelm, S., *Cognitive Behavioral Therapy for OCD* (MGH Psychiatry Academy presentation); Crichton, N., *Understanding and Treating Resistance in OCD Through EMDR* (presentation); and Smith, A., *OCD Treatment Masterclass: When Clients Are Stuck in Loops of Intrusive Thoughts and Compulsive Rituals* (eCare Behavioral Health Institute, recorded webinar). Not affiliated with or endorsed by these authors or publishers.

Additional resources for clients: International OCD Foundation (iocdf.org).

SECTION 16

16 Glossary

Key terms in plain language.

Accommodation. When others change their behavior to reduce a person's OCD distress, such as answering reassurance questions or joining rituals.

Avoidance. Staying away from triggers to prevent distress; it works like a compulsion and keeps OCD going.

Compulsion. A physical or mental action done to reduce distress, prevent a feared outcome, or feel "just right."

ERP. Exposure and Response Prevention: facing triggers on purpose while not doing compulsions.

Exposure ladder. A ranked list of triggering situations used to plan exposures, also called a hierarchy.

Extinction burst. A temporary spike in OCD's demands when rituals or accommodation stop, usually a sign the change is working.

Imaginal exposure. Facing a feared thought or outcome through a written or recorded script.

Intrusive thought. An unwanted thought, image or urge that pops into the mind. Nearly everyone has them.

Lapse. A temporary slip back into old habits; different from a full relapse.

Mental ritual. A compulsion done in the mind, such as reviewing, counting, praying or replacing thoughts.

Neutralizing. Trying to cancel out a bad thought, often with a good thought, phrase or action.

Obsession. A repeated, unwanted thought, image, urge or doubt that causes distress.

Reassurance seeking. Asking others, searching or confessing to feel certain or relieved.

Response prevention. Choosing not to perform compulsions, avoidance or reassurance seeking.

Safety behavior. A subtle action that reduces distress during exposure, such as using a tissue on a door handle.

Scripting. Thought exposure: repeating a short version of a feared thought, or "maybe, maybe not," until it loses its punch.

SUDs. Subjective Units of Distress, a 0 to 100 rating of how distressed you feel.

Thought-action fusion. The belief that thinking something makes it more likely or is morally equal to doing it.

Uncertainty tolerance. The ability to live with not knowing for sure; a core skill built through ERP.

Y-BOCS. Yale-Brown Obsessive Compulsive Scale, a standard clinical measure of OCD severity.